INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Crizotinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00243
Version 7

Continuous oral treatment

Treatment stages

01 **Continuous oral crizotinib**
Daily treatment; no rest week

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Crizotinib targets abnormal ALK or ROS1 signalling in selected advanced non-small-cell lung cancers. The aim is to control cancer and delay progression; the choice depends on the tumour test, previous treatment, other options and your priorities.

ORAL TARGETED THERAPY: ALK/ROS1 TYROSINE KINASE INHIBITOR

Crizotinib

Blocks growth signals from abnormal ALK or ROS1 proteins

Validated ALK or ROS1 tumour test → Crizotinib blocks the abnormal signal → Cancer growth may slow

An ALK or ROS1 rearrangement can drive cancer growth. Crizotinib blocks these signals in suitable cancers. It can also affect healthy tissues, so regular tests and symptom reporting remain important.

Report changes in breathing, vision, heartbeat, swelling, liver symptoms or severe bowel symptoms promptly. Check all other medicines before starting or changing them.

Your treatment and its purpose

Confirm your indication

Adults with advanced ALK-positive or ROS1-positive non-small-cell lung cancer (NSCLC). NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 v7 · official regimen

Duration and review

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

NCCP 00243 v7 · official regimen

ALK and ROS1 evidence: different study populations

Study results describe groups and cannot predict an individual outcome.

No cross-trial ranking or individual survival estimate. Confirm contemporary options with the team.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Crizotinib checks over time

Columns show sequence, not equal time periods. The team books tests and reviews results. NCCP “electrocardiogram (ECG) every 2 cycles” does not define an interval for this continuous oral treatment; agree the actual cadence. No patient-entered dates are needed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00243 v7 · official regimen · EMA Xalkori product information: adult NSCLC and patient leaflet

THE WHOLE TREATMENT JOURNEY

Crizotinib monotherapy: treatment and checks

Your treatment pattern

Continuous oral treatment

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Adult ALK-positive or ROS1-positive advanced NSCLC

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 2 months	After month 2	Ongoing benefit review	Hold / change / stop
Crizotinib treatment NCCP 00243 · SMPC	Confirm adult indication, prescription and formulation	Daily treatment; no planned rest week	Continue authorised daily schedule	Review cancer benefit and tolerability	Confirm whether held, changed or stopped
Liver tests NCCP 00243 · SMPC	Baseline alanine aminotransferase (ALT), aspartate aminotransferase (AST) and bilirubin	Weekly for first 2 months (SmPC); reconcile NCCP fortnightly wording	Monthly; more often if abnormal	Continue checks and symptom review	Review recovery before any authorised restart
Blood count and kidneys NCCP 00243 · SMPC	Baseline full blood count (FBC) and renal profile	Monthly; earlier if fever, illness or dehydration	Monthly and when clinically needed	Review renal function and other medicines	Follow unresolved abnormalities
Heart and lungs NCCP 00243 · SMPC	ECG/ electrolytes near first dose; chest imaging and symptoms	Periodic ECG/ electrolytes; pulse/blood pressure (BP) and symptoms	Agree explicit ECG cadence; assess new symptoms promptly	NCCP chest X-ray monthly; disease imaging per consultant	Investigate breathlessness, fainting, swelling or rhythm symptoms

Care / check	Before starting	First 2 months	After month 2	Ongoing benefit review	Hold / change / stop
Vision, medicines and daily life NCCP 00243 · SMPC	Review existing vision symptoms and every medicine/ supplement	Report visual change; check nausea support and adherence	Symptoms, interactions, food and supply reviewed	Persistent/ worsening vision symptoms need eye review	Severe visual loss needs urgent assessment; confirm medicines/ follow-up

What each check is for

1 Eligibility and safe start

Before first dose

Correct biomarker/line and organ/cardiac context are essential.

Ask / **act:** Confirm prescription, contraception/feeding plan, interaction review and contacts.

2 Early liver surveillance

Weekly for first 2 months

Clinically important liver injury can develop early.

Ask / **act:** Arrange tests, identify reviewer and repeat sooner for abnormalities.

3 Monthly and symptom-led checks

Throughout treatment

Continuous tablets still require continuing clinical review.

Ask / **act:** Review benefit, adverse events, interactions and supply.

4 Hold or stop safely

Toxicity, progression or changed priorities

Dose changes can alter both capsule strength and frequency.

Ask / **act:** Give explicit new instructions and follow-up; only the prescriber authorises restart.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily treatment, monitoring and next decisions

No personal dates are needed. Confirm actual tests and appointments with your team.

Treatment or check	Timing in this view
Preparation	**Before starting** Confirm biomarker, indication, baseline tests and medicines
Daily capsules	**First 2 months → After month 2 → Ongoing benefit review** Continuous oral treatment at the prescribed strength and frequency
Tests and symptoms	**First 2 months → After month 2 → Ongoing benefit review → Hold / change / stop** Weekly early liver tests, then monthly; ongoing blood/heart checks and urgent symptoms
Next plan	**Hold / change / stop** Clinician authorises continued treatment, hold, change or stop

The starting dose and any reduced dose may have different frequencies.

Columns are milestones, not equal time periods.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Taking crizotinib at home

The prescription and capsule formulation determine how you take it.

Taking, missing or vomiting a dose

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time.

When a dose changes

A dose may change from twice daily to once daily. Ask for a written strength-and-frequency plan. Do not combine old and new instructions, add missed doses or restart after a clinical hold without advice.

NCCP 00243 v7 · official regimen

Tests and symptom reviews

Baseline checks and ongoing monitoring do different jobs.

Monitoring plan

Before starting: confirm the tumour test, blood count, kidney/liver tests, chest imaging, symptoms and all medicines; ECG and electrolytes near the first dose. Liver tests are planned weekly for the first 2 months using current product information, then monthly; NCCP says every 2 weeks initially, so confirm the local plan. Blood count and kidney tests are monthly. ECG, electrolytes, pulse and blood pressure need a planned schedule and earlier review if unwell. NCCP lists monthly chest X-ray; other disease imaging follows your consultant plan.

Eyes and breathing

Tell your team about visual symptoms and any new cough or breathlessness. Persistent or worsening vision problems need an eye assessment; severe visual loss needs urgent review. The routine paediatric eye-examination timetable does not apply automatically to adult NSCLC.

Support during treatment

Ask for help with eating, fatigue, travel, work and emotional wellbeing.

A tailored anti-sickness plan

Crizotinib has moderate-to-high oral sickness risk. Agree daily prevention and rescue treatment with your team. The chosen medicine must account for heart-rhythm and drug-interaction risks; do not add anti-sickness medicines or a prolonged steroid course yourself.

NCCP 00243 v7 · official regimen · NCCP systemic anticancer therapy (SACT) nausea/vomiting classification V6, oral Tables 5 and 8 · NCCP NCIS Medical Oncology antiemetics V8 (2025)



Food and medicine interactions

Avoid grapefruit/grapefruit juice and St John’s wort. Check prescription medicines, pharmacy medicines, vitamins and herbal products before taking them. Some antibiotics, antifungals, heart medicines and seizure treatments interact. Do not stop a prescribed medicine without a plan.

Doses and supply

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time. Request repeat supply early and keep a current medicines list. A clinician-held dose needs clinician authorisation to restart.

Pregnancy, fertility and feeding

Discuss fertility preservation before treatment. Use an effective agreed contraception plan during treatment and for at least 90 days after the last dose; this applies to men and women. Oral contraceptives may be less effective. Tell the team promptly about pregnancy and avoid breastfeeding during treatment.

Vision, driving and sunlight

Report visual changes. Take care with driving or machinery if vision, dizziness or fatigue affects safety. Avoid prolonged sun exposure and use protective clothing and sunscreen.

Symptoms that need prompt help

Keep the oncology contact number available day and night.

Do not wait for the next visit

Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

When you call

Give the medicine name, when the last dose was taken, what has changed, and whether you can drink and pass urine. Do not delay a call to collect details.

Discuss benefit and choices

In separate ALK studies, crizotinib delayed progression compared with the chemotherapy used at that time. The ROS1 study showed cancer responses but had no comparison group. These studies involve different populations and cannot rank present-day medicines or predict an individual outcome. Ask how the evidence fits your tumour, previous therapy, brain involvement and current alternatives.

Evidence depends on the setting

ALK first-line, ALK after previous treatment and ROS1 disease were studied separately. The chemotherapy comparisons are historical; the ROS1 study was small and had no control group. Ask about the alternatives suitable for you, including supportive care and choosing not to proceed.

PROFILE 1007: previously treated ALK NSCLC (2013) · PROFILE 1014: first-line ALK NSCLC (2014) · PROFILE 1014 final OS (2018) · PROFILE 1001 ROS1 update, including OS (2019)

A change in the plan

Reviews consider symptoms, tests, cancer response, side effects and your priorities. Ask who will arrange follow-up and how to report symptoms after treatment stops. You may reconsider your consent.

Practical support

A tailored anti-sickness plan

Food and medicine interactions

Doses and supply

Pregnancy, fertility and feeding

Vision, driving and sunlight

Taking tablets / attending treatment

Questions to ask

Do I need to enter dates?

No. This map gives general milestones. Your team books actual appointments and authorises treatment changes.

Do side effects mean it is working?

Side effects do not show whether cancer is responding. The team uses symptoms, examinations, imaging and other planned tests to assess benefit.

Explain the plan back: Can you explain your capsule strength and frequency, the 6-hour missed-dose rule, the next liver test, medicines to check, and when to call urgently?

Adult advanced non-small-cell lung cancer (NSCLC) · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Crizotinib daily oral treatment

Active medicines: crizotinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individually selected oral antiemetic prophylaxis	Prescriber/pharmacy to select and prescribe under local oral-systemic anticancer therapy (SACT) policy	Daily during oral treatment, reviewed for ongoing need and tolerability NCCP V6 Table 5 supports daily oral 5-HT3 antagonist prophylaxis for moderate-high oral risk; crizotinib can prolong QT and slow heart rate. Resolve electrocardiogram (ECG)/electrolyte and interaction risks before selecting the agent/dose. Do not copy a parenteral Day 1–4 steroid/NK1 regimen.

Additional phase-specific support

- Crizotinib has moderate-to-high oral sickness risk. Agree daily prevention and rescue treatment with your team. The chosen medicine must account for heart-rhythm and drug-interaction risks; do not add anti-sickness medicines or a prolonged steroid course yourself.
- Avoid grapefruit/grapefruit juice and St John’s wort. Check prescription medicines, pharmacy medicines, vitamins and herbal products before taking them. Some antibiotics, antifungals, heart medicines and seizure treatments interact. Do not stop a prescribed medicine without a plan.
- Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time. Request repeat supply early and keep a current medicines list. A clinician-held dose needs clinician authorisation to restart.
- Discuss fertility preservation before treatment. Use an effective agreed contraception plan during treatment and for at least 90 days after the last dose; this applies to men and women. Oral contraceptives may be less effective. Tell the team promptly about pregnancy and avoid breastfeeding during treatment.
- Report visual changes. Take care with driving or machinery if vision, dizziness or fatigue affects safety. Avoid prolonged sun exposure and use protective clothing and sunscreen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Adult advanced non-small-cell lung cancer (NSCLC) · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. National Cancer Control Programme (NCCP) 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 · VERSION 7

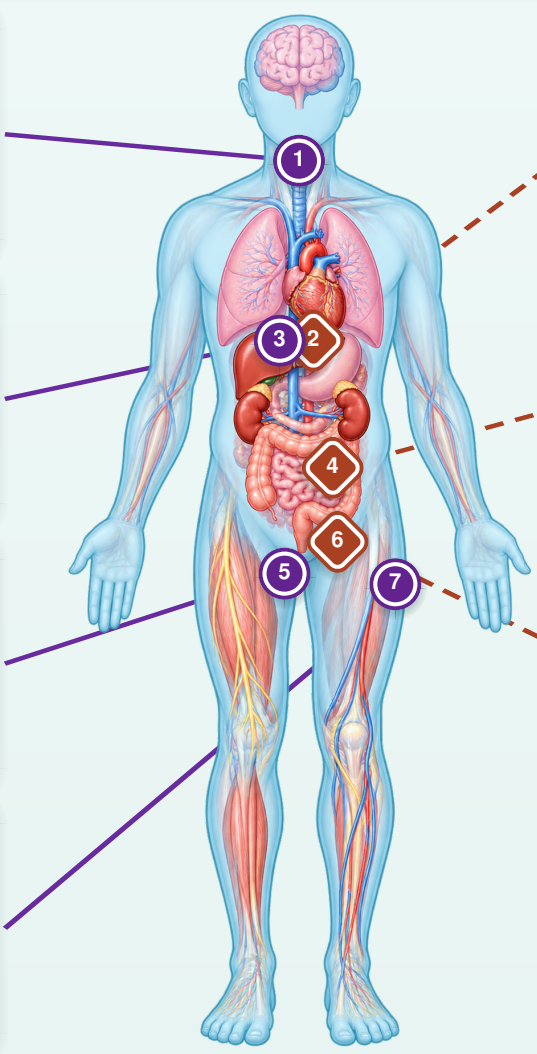
Crizotinib monotherapy

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Oral ALK/ROS1 targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Vision

Crizotinib

Report visual change; severe visual loss needs urgent assessment.

2 Heart and rhythm

Crizotinib

Slow pulse, rhythm changes or fluid-related swelling need assessment.

3 Breathing

Crizotinib

New cough or breathlessness can indicate serious lung inflammation.

4 Liver

Crizotinib

Liver tests and prompt reporting of jaundice or dark urine protect against serious injury.

5 Bowel and stomach

Crizotinib

Nausea, vomiting, bowel changes and severe abdominal pain need advice.

6 Kidneys and hydration

Crizotinib

Report poor drinking or reduced urine; kidney tests continue during treatment.

7 Skin and sunlight

Crizotinib

Rash and sun sensitivity can occur; use protection and report severe changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Vision changes	☐	☐	☐	
Nausea	☐	☐	☐	
Diarrhoea	☐	☐	☐	
Constipation	☐	☐	☐	
Swelling	☐	☐	☐	
Fatigue	☐	☐	☐	
Breathlessness / cough	☐	☐	☐	
Palpitations / fainting	☐	☐	☐	
Skin / sun sensitivity	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/ shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Emergency symptoms: 112 / 999.

Carry your treatment details

Adult advanced NSCLC · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. NCCP 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Crizotinib monotherapy

NCCP 00243 v7

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact your oncology team immediately for new or worsening breathlessness or cough, fainting, palpitations, severe dizziness, rapidly increasing swelling/weight, sudden or severe loss of vision, yellow skin/eyes, dark urine, fever/shivering or sudden illness, severe abdominal pain, or persistent vomiting/diarrhoea with poor drinking or little urine. Do not wait for the next appointment. Severe breathing difficulty, collapse or severe chest pain needs emergency help.

https://sactcheck.com/patient/00243/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00243 v7 · pNCCP00243 p4; classification Tables 5/8; NCIS V8 p4 QT caution**
   https://healthservice.hse.ie/documents/6934/243_V7_Crizotinib_Monotherapy.pdf
2. **EMA Xalkori product information: adult non-small-cell lung cancer (NSCLC) and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/xalkori-epar-product-information_en.pdf
3. **NCCP systemic anticancer therapy (SACT) nausea/vomiting classification V6, oral Tables 5 and 8**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS Medical Oncology antiemetics V8 (2025)**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **PROFILE 1007: previously treated ALK NSCLC (2013)**
   https://pubmed.ncbi.nlm.nih.gov/23724913/
6. **PROFILE 1007 final OS: 2016 primary conference abstract**
   https://ascopubs.org/doi/10.1200/JCO.2016.34.15_suppl.9066
7. **PROFILE 1014: first-line ALK NSCLC (2014)**
   https://pubmed.ncbi.nlm.nih.gov/25470694/
8. **PROFILE 1014 final OS (2018)**
   https://pubmed.ncbi.nlm.nih.gov/29768118/
9. **PROFILE 1001 ROS1 update, including OS (2019)**
   https://pubmed.ncbi.nlm.nih.gov/30980071/